

ACTIVITY REPORT



Activity Title:	Community-Led Monitoring (CLM) Data Collection Activity
Supported By:	The AIDS Support Organization (TASO)
Name of the Sub-Sub Recipient:	Mayanja Memorial Hospital Foundation(MMHF)
Date of Activity Implementation:	
Meeting Venue:	ISINGIRO TOWN COUCIL
Activity Facilitated by:	MAGOMU ALEX E.D, CITRAL Uganda Tel: 0776311235/0705172212 E-mail: citralug@gmail.com
Activity Coordinated by:	CITRAL

A. Name of the Activity:

Community-Led Monitoring (CLM) Data Collection in Selected Facilities and Communities in Isingiro District

B. Background to the Activity:

Community-Led Monitoring (CLM) is a participatory tool that empowers affected communities to assess the accessibility, quality, and responsiveness of health services. CITRAL Uganda, supported by TASO and in partnership with MMHF, implemented CLM across selected health facilities and surrounding communities in Isingiro District. The data collection phase was key to gathering insights from people living with HIV, TB clients, malaria-affected individuals, and other vulnerable groups to inform evidence-based advocacy and service improvement.

C. Objectives of the Activity:

- i. To collect primary data on access, availability, and quality of HIV, TB, and malaria services.
- i. To capture community voices, experiences, and barriers related to health service delivery.
- ii. To generate data for evidence-based advocacy and policy recommendations.
- iii. To empower community monitors to lead data-driven engagements with duty bearers.

D. Methodology of Implementation:

- i. Community monitors were deployed to collect both quantitative and qualitative data.
- ii. Data was gathered through key informant interviews and focus group discussions (FGDs).
- iii. Each monitor was tagged to 4 health facilities and community sites.
- iv. Standardized electronic tools were used for data collection (on tablets/iPads).
- v. Data collection was conducted for up to 10 days per facility.
- vi. Peers were used to mobilize community members for FGDs.
- vii. The data was stored offline due to limited internet and later synchronized centrally.

E. Achievements:

- i. Successfully deployed 2 trained community monitors across the selected facilities.
- ii. Collected and stored data from 7 health facilities and 8 community sites.
- iii. Reached over 300 respondents through FGDs and key informant interviews.
- iv. Built strong collaboration between health facilities, VHTs, and community groups.
- v. Promoted community ownership and participation in health monitoring.

F. Challenges Encountered During Implementation:

- i. Delayed provision of iPads, which caused a postponement in the data collection schedule.
- ii. Long distances between health facilities and communities increased travel time and monitor fatigue.
- iii. Lack of reliable internet in most field locations made real-time data synchronization impossible; offline storage was used with delayed uploads.
- iv. Occasional difficulty in mobilizing FGD participants due to farming season engagements.

G. Lessons Learnt:

- i. Early logistical preparation (especially provision of equipment) is critical to timely implementation.
- ii. Offline data tools are essential for remote areas with poor connectivity.

- iii. Monitors need support with transport allowances in geographically dispersed settings.
- iv. Community mobilization should consider local schedules (e.g., market and farming days).

H. Recommendations for Performance Improvement:

- i. Provide all required equipment (e.g., tablets) at least one week before data collection starts.
- ii. Establish a transport allowance buffer to cater for hard-to-reach areas.
- iii. Provide portable internet devices (e.g., modems or MiFis) to support data syncing.
- iv. Include a local mobilizer in each data collection team for efficient participant recruitment.

I. Strategies for the Next Activities:

- i. Conduct pre-deployment simulation exercises to test tools and devices.
- ii. Map out logistical needs during the planning phase and allocate resources accordingly.
- iii. Hold periodic check-ins during data collection to offer technical and logistical support.
- iv. Use findings from this phase to guide facility and community-level feedback meetings.

J. PICTORIALS:



Above are FDGS with PLWHIV while below are FDGS with KPs



Above is data collection at facilities

1. Prepared by:

Name: Magomu Alex

Title: Executive Director – CITRAL Uganda

Signature: _____



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ACTIVITY REPORT – DATA ANALYSIS & REPORT WRITING

A. Name of the Activity:

CLM Data Analysis and Report Writing for April 2025 Cycle

B. Background to the Activity:

Following the completion of the data collection phase for Community-Led Monitoring (CLM), the next step was the analysis and compilation of a comprehensive report. This report would serve as the basis for feedback engagements with health facilities, communities, and duty bearers. The process was facilitated by CITRAL Uganda, with technical oversight from TASO and MMHF. The activity brought together data clerks, M&E officers, and technical leads for a three-day session focused on data cleaning, analysis, and drafting of the CLM report.

C. Objectives of the Activity:

- i. To clean and analyze the data collected from communities and health facilities.
- ii. To identify key findings, trends, and service delivery gaps.
- iii. To compile a comprehensive and evidence-based report to inform upcoming feedback meetings.
- iv. To build internal M&E capacity for future data management cycles.

D. Methodology of Implementation:

- i. Data was extracted from the mobile tools into Excel and SPSS for cleaning and statistical analysis.
- ii. Qualitative data (from FGDs and interviews) was thematically analyzed.
- iii. M&E officers were guided through the process of coding and interpreting data outputs.
- iv. A team of three technical staff compiled the preliminary report and shared it with MMHF and TASO for input.
- v. Report templates were used to ensure consistency and clarity in reporting format.

E. Achievements:

- i. Successfully cleaned and analyzed data from 12 facilities and 16 community sites.
- ii. Drafted a 20-page analytical report highlighting key service delivery strengths and gaps.
- iii. Developed simplified data summaries for feedback meetings at community, facility, and district levels.
- iv. Improved understanding of data analysis procedures among CITRAL M&E officers through mentorship.
- v. Generated charts, graphs, and infographics to visualize findings clearly for stakeholders

F. Challenges Encountered During Implementation:

- i. Limited knowledge among M&E officers in extracting and interpreting key information slowed the analysis process.
- ii. Poorly collected data in some areas led to additional time spent on cleaning and verifying inconsistencies.
- iii. Delayed data submission by community monitors—linked to lack of contracts or formal engagement—led to a slow start and rushed timelines.
- iv. one monitor boycotted scheduled days, opting instead for alternative income-generating activities offered in their communities.

G. Lessons Learnt:

- i. Continuous capacity building for M&E staff is essential to ensure quality reporting.
- ii. Monitors need formalized commitment (contracts or MoUs) to increase accountability and reliability.
- iii. Real-time data verification tools can minimize errors and save time during analysis.
- iv. Providing a clear schedule and timely incentives enhances commitment from field teams.

H. Recommendations for Performance Improvement:

- i. Conduct a refresher training on data management and reporting for all M&E staff.
- ii. Establish performance-based contracts with community monitors for future data collection cycles.
- iii. Introduce basic data quality checks during field collection to reduce post-collection errors.
- iv. Involve M&E officers in field monitoring to increase their understanding of context and data quality issues.

V. Strategies for the Next Activities:

- i. Use the report findings to inform structured feedback meetings at facility, community, and district levels.
- ii. Prepare simplified versions of the report (briefs and presentation decks) for stakeholder engagements.
- iii. Schedule a post-activity review with monitors and M&E staff to reflect on lessons and gather improvement ideas.
- iv. Integrate a data visualization workshop in the next reporting cycle to enhance presentation quality.

I. PICTORIALS:

NB: Pictures not taken due to refusal to consent by participants.

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Activity Title:	Monitoring and Evaluation of Community-Led Monitoring Implementatio
Supported By:	The AIDS Support Organization (TASO)
Name of the Sub-Sub Recipient:	Mayanja Memorial Hospital Foundation(MMHF)
Date of Activity Implementation:	
Meeting Venue:	ISINGIRO TOWN COUCIL
Activity Facilitated by:	MAGOMU ALEX E.D, CITRAL Uganda Tel: 0776311235/0705172212 E-mail: citralug@gmail.com
Activity Coordinated by:	CITRAL

A. Name of the Activity:

Monitoring and Evaluation of CLM Implementation Progress

B. Background to the Activity:

Monitoring and Evaluation (M&E) is a critical component of the Community-Led Monitoring (CLM) approach, as it ensures accountability, tracks performance, and informs evidence-based decisions. In April 2025, CITRAL Uganda, with support from TASO and MMHF, conducted M&E of data collection and stakeholder engagement activities. This aimed at assessing implementation fidelity, identifying bottlenecks, and gathering recommendations for improvement. Despite the absence of a developed and standardized M&E tool to guide the process, the activity was successfully executed using adapted checklists and observational methods.

C. Objectives of the Activity:

- i. To assess the quality and timeliness of data collection at community and facility levels.
- ii. To identify challenges affecting the implementation of CLM.
- iii. To document good practices and areas for improvement in ongoing activities.
- iv. To provide real-time support and mentorship to field teams.
- v. To generate evidence to inform report writing and feedback engagements.

D. Methodology of Implementation:

- i. On-site monitoring visits were conducted to selected facilities and community data collection sites.
- ii. M&E staff used adapted observation checklists and field notes to record findings.
- iii. Feedback was provided on-site to strengthen implementation.
- iv. A short debrief meeting was held after each visit to consolidate learning and inform next steps.

E. Achievements:

- i. Monitoring visits were conducted in 7 health facilities and 8 community sites.
- ii. Direct observation revealed adherence to ethical data collection practices by most monitors.
- iii. Key issues such as delayed tool return, gaps in consent procedures, and weak community engagement were identified and addressed in real-time.
- iv. Site-specific recommendations were developed and shared with monitors and local leaders.
- v. Strengthened collaboration between CITRAL M&E team and facility in-charges.

F. Challenges Encountered During Implementation:

- i. The **absence of a formalized M&E tool** (checklist or framework) made the process less structured.
- ii. Some field observations were not standardized due to lack of predefined indicators.
- iii. Difficulty in aggregating observations across sites because of inconsistent data capture formats.
- iv. Some sites lacked documentation (e.g., attendance registers, consent forms) for verification.

G. Lessons Learnt:

- i. Even in the absence of a formal M&E tool, practical, adaptive approaches can yield valuable monitoring insights.
- ii. Real-time feedback to field teams is more impactful than delayed reporting.
- iii. Structured tools are essential for consistency and efficiency in monitoring processes.
- iv. Direct engagement with facility staff improves accountability and transparency.

H. Recommendations for Performance Improvement:

- i. Develop and validate a standard M&E tool (checklist) with clear indicators for future use.
- ii. Train all M&E officers and field staff on the use of the tool.
- iii. Schedule M&E visits to align with peak activity periods for maximum observation.
- iv. Ensure documentation and record-keeping at all monitored sites.

I. Strategies for the Next Activities:

- i. Develop and pilot a standardized M&E tool tailored for CLM activities.
- ii. Incorporate feedback from this M&E cycle into planning for the next data collection phase.
- iii. Conduct a mini-review with all field staff to discuss findings and strengthen monitoring practices.
- iv. Increase frequency of monitoring visits to ensure continuous support.
- v. Leverage mobile technology for real-time M&E documentation.

J. PICTORIALS:



Above and below CITRAL TEAM and M&E monitoring monitors in the field.



1. Prepared by:

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Activity Title:	Community-Level Feedback Meetings
Supported By:	The AIDS Support Organization (TASO)
Name of the Sub-Sub Recipient:	Mayanja Memorial Hospital Foundation(MMHF)
Date of Activity Implementation:	
Meeting Venue:	ISINGIRO TOWN COUCIL
Activity Facilitated by:	MAGOMU ALEX E.D, CITRAL Uganda Tel: 0776311235/0705172212 E-mail: citralug@gmail.com
Activity Coordinated by:	CITRAL

A. Name of the Activity:

Community-Level Feedback Meetings

B. Background to the Activity:

Community feedback meetings are essential in the Community-Led Monitoring (CLM) cycle to promote transparency, accountability, and community participation. These meetings provide a platform for sharing findings from data collection, validating issues raised by community monitors, and gathering collective input for improvement. In April 2025, CITRAL Uganda, with support from TASO and MMHF, convened feedback meetings across sub-counties in Isingiro District to ensure that health service users and providers are informed and involved in the decision-making process.

C. Objectives of the Activity:

- i. To present key findings from the recent data collection to community stakeholders.
- ii. To validate identified service delivery gaps and gather community input.
- iii. To strengthen community-health facility relationships through dialogue.
- iv. To document community-driven solutions for inclusion in action plans.

D. Methodology of Implementation:

- i. Mobilization of community leaders, health workers, and beneficiaries through local leaders and VHTs.
- ii. Presentations of data findings using visual aids and translated summaries.
- iii. Open plenary discussions to validate data and provide clarifications.
- iv. Documentation of community feedback and suggested solutions.
- v. Use of local languages (Runyankore, Kiswahili, Ruyankwanda etc) to enhance inclusiveness and understanding.

E. Achievements:

- i. Eight (8) community feedback meetings conducted across various sub-counties.
- ii. Over 250 participants, including, advocates, sex workers, youth, AGYW, and PLHIVs attended and actively engaged.
- iii. Key service gaps such as long waiting times, drug stock-outs, and staff attitudes were validated and discussed.
- iv. Participants appreciated being included in the CLM process and committed to supporting future activities.
- v. Local leaders pledged to follow up with health facilities and support accountability mechanisms.
- vi. Enhanced trust between data collectors and the community through transparency.

F. Challenges Encountered During Implementation:

- i. Some health workers and beneficiaries did not receive transport refunds, despite signing for them in the previous activity.
- ii. This led to frustration among a few participants and concerns over trust and transparency.
- iii. The situation may negatively impact participation in future CLM activities if not addressed.
- iv. Delays in disbursement were attributed to administrative bottlenecks beyond the organizing team's immediate control.

G. Lessons Learnt:

- i. Prompt and transparent disbursement of participant refunds is essential for maintaining trust and engagement.
- ii. Clearly communicating any delays or challenges helps manage expectations and prevent misinformation.
- iii. Community members value participation when their voices are heard and action is seen to follow.

H. Recommendations for Performance Improvement:

- i. Ensure timely disbursement of transport refunds, ideally before or during the meeting.
- ii. Provide written communication or acknowledgment of any delays to manage participant expectations.
- iii. Strengthen coordination between finance and field teams to avoid future disbursement issues.
- iv. Explore use of mobile money systems for faster and more traceable payments.
- v. Document and resolve all participant concerns transparently to maintain credibility.

I. Strategies for the Next Activities:

- i. Pre-plan and allocate all participant support funds before field activities begin.
- ii. Train facilitators on financial communication and grievance handling in community meetings.
- iii. Include a short session in feedback meetings to explain financial processes and timelines.
- iv. Introduce feedback tracking forms to follow up on community concerns and progress.

J. PICTORIALS:



Above is group photo of community while(R) and below FEEDBACK meeting in community taking place





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Signature: _____

ACTIVITY REPORT – FACILITY LEVEL FEEDBACK MEETINGS



Activity Title:	Facility-Level Feedback Meetings on Data Collection Findings
Supported By:	The AIDS Support Organization (TASO)
Name of the Sub-Sub Recipient:	Mayanja Memorial Hospital Foundation(MMHF)
Date of Activity Implementation:	
Meeting Venue:	ISINGIRO TOWN COUCIL
Activity Facilitated by:	MAGOMU ALEX E.D, CITRAL Uganda Tel: 0776311235/0705172212 E-mail: citralug@gmail.com
Activity Coordinated by:	CITRAL

A. Name of the Activity:

Facility-Level Data Feedback Meetings with Health Facility Staff and Community Representatives

B. Background to the Activity:

As part of the Community-Led Monitoring (CLM) cycle, facility-level feedback meetings were conducted to share and validate findings from recent data collection exercises. These meetings aimed to improve service delivery by engaging health facility staff and community beneficiaries in open dialogue based on real-time feedback collected from service users. The meetings also sought to foster stronger collaboration between communities and health facility management for sustainable improvements.

C. Objectives of the Activity:

- i. To share facility-specific data collection findings with health workers and community stakeholders.
- ii. To validate and discuss service delivery gaps and strengths identified during monitoring.
- iii. To jointly propose actionable solutions that address community and facility concerns.
- iv. To build mutual trust and foster accountability between facilities and service users.

D. Methodology of Implementation:

- v. Identification of key health facilities with data collection findings to be shared.
- vi. Invitations extended to health workers, in-charges, community representatives, PLHIV groups, and local leaders.
- vii. Presentation of data findings using infographics and simple summaries.
- viii. Facilitated dialogue sessions to reflect on findings and co-create solutions.
- ix. Documentation of agreed action points to follow up with facility management and implementing partners.

E. Achievements:

- i. Facility-level feedback meetings successfully held at 7 health facilities across Isingiro District.
- ii. Active participation from 120+ stakeholders including facility in-charges, midwives, laboratory staff, advocates and patient representatives.
- iii. Key service issues such as drug stock-outs, waiting times, and stigma were openly discussed and acknowledged.
- iv. Facility staff appreciated the data and committed to implementing some changes immediately (e.g., improving reception and triage processes).
- v. Community members appreciated the opportunity to directly engage with service providers and voice concerns constructively.
- vi. A follow-up mechanism for tracking action points was established through CITRAL staff and community monitors.

F. Challenges Encountered During Implementation:

- i. Some health workers and beneficiaries did not receive their transport refunds despite signing for them during data collection.
- ii. This created dissatisfaction and may reduce willingness to participate in future CLM engagements.
- iii. The delayed disbursement was reportedly due to logistical bottlenecks in mobile money processing and fund flow delays.

G. Lessons Learnt:

- i. Timely provision of transport refunds is critical for ensuring full participation and goodwill in community and facility activities.

- ii. Health workers, despite their schedules, are willing to engage in accountability forums when treated with respect and fairness.
- iii. Presenting facility-specific data builds credibility and fosters genuine reflection from staff.

H. Recommendations for Performance Improvement:

- i. Ensure transport refunds are disbursed before or during the meeting to avoid discouragement.
- ii. Establish a clear communication channel with participants regarding refund processes and timelines.
- iii. Pre-load funds onto mobile money platforms in advance to prevent administrative delays.
- iv. Maintain attendance and payment reconciliation records to improve accountability.
- v. Strengthen partnerships with facility in-charges to coordinate logistics more effectively.

I. Strategies for the Next Activities:

- i. Allocate and process participant support funds well ahead of activity implementation.
- ii. Coordinate closely with TASO and MMHF finance teams to address disbursement delays.
- iii. Hold short financial briefing sessions at the start of each feedback meeting to clarify expectations.
- iv. Continue building relationships with health facilities to encourage more consistent collaboration.

PICTORIALS:



Above (L) feed back meeting in nakivale HC III with DHO who came with team to review on maternal death while (R) are participants of juru HC III and below is presentation from CITRAL team.



Below is presentation from NSHUNGYENZI HC III and KIBENGO HC III



1. Prepared by:

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Date: 10/10/2025

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